MEDICAL FAX

FROM: Memorial Hospital of Converse County
(307) 316-0397
TO: (503) 493-6789
SUBJECT: Untitled
DATE: Tue, 04/30/24, 2:52 PM MDT
PAGES: 22 (not including coversheet)

INSTRUCTIONS: ALL RECORDS FROM REQUESTED DATE RANGE

ITEMIZED BILLING WILL BE SENT ASAP

INVOICE INCLUDED

This fax transmission is for the sole use of the intended recipient(s) and contains confidential information. Any unauthorized review, use, disclosure or distribution is strictly prohibited. If you are not the intended recipient, please immediately contact the sender.

Having trouble reading this fax?
You can view it online at
http://memorialhospitalofconversecounty.myupdox.com by entering the following information:
Fax ID: **3d9795e**
Password: **Eyes-001-Popsicle**
This fax expires on May 5, 2024 2:52:04 PM. It will not be available online after that time.

BJSF
BRANDON J. SQUIRES
LAW FIRM

Kristine McKibben, Paralegal
5200 Meadows Rd. Ste. 150
Lake Oswego, OR 97035
Phone: 503-493-6700 Fax: 503-493-6789
kristine@squiresfirm.com

April 30, 2024

Via Fax Only to: 1-307-358-7355
Attn: Medical Records
Memorial Hospital of Converse County
111 South Fifth
PO Box 1450
Douglas, WY 82633

Please Expedite

Re:	My Client:	Viktoriya Pulliam
	DOL:	2/11/2020
	DOB:	12/6/1989

To Whom it May Concern,

This office represents the above-mentioned client who was injured in an incident which occurred on the above-mentioned date. **The purpose of this letter is to request:**

1. A COMPLETE COPY OF YOUR ENTIRE FILE FROM JULY 19, 2023 TO THE PRESENT, including copies of all medical records, chart notes, x-ray reports, radiology images, test results, letter reports, consultative reports, correspondence, transcribed records, laboratory/test results, patient questionnaires, etc. which you might have regarding this patient. The HIPAA Privacy Rule does not prohibit the use, disclosure, or request of an entire medical record. **Please provide electronic copy when available by disk, fax to 503-493-6789, or e-mail to: kristine@squiresfirm.com.**

2. A COMPLETE COPY OF YOUR ITEMIZED BILLING STATEMENT OR BILLING LEDGER showing the charges for all treatment provided to this patient from February 11, 2020, to the present.

A signed HIPAA compliant Medical Authorization to Disclose Protected Health Information is enclosed/attached for your records. If there is any charge for the reproduction of these documents, please submit your statement to this office and we will promptly remit payment.

Thank you.

Kind Regards,

/s/ Kristine McKibben

Kristine McKibben
Paralegal to Brandon J. Squires

Encl. Medical Authorization

MEDICAL AUTHORIZATION TO USE AND DISCLOSE PROTECTED HEALTH INFORMATION
(RELEASE OF MEDICAL RECORDS)

I [patient/client] Viktoryah Pullman DOB 12/6/89 authorize [provider's name]:
Memorial Hospital of Converse County to disclose health information / release a copy of my protected health information to my attorneys at the law office of **The Brandon J. Squires Law Firm, 5200 Meadows Rd., Ste. 150, Lake Oswego, OR 97035.** The information will be used, at my request, to process my claim for injuries resulting from a personal injury and/or motor vehicle accident. This information may include pre-existing conditions that were aggravated by, affected by or contributed to my injury or injuries.

By initialing the spaces below, I specifically authorize the release/disclosure of the following health information/medical records, if such records exist:

VP **THE ENTIRE MEDICAL RECORD (any and all information including, but not limited to, medical billings, physician reports, history & physical exams, chart notes, laboratory results, x-ray or other imaging reports, and prior records)** to the above-named recipient. The recipient understands this record may be voluminous and agrees to pay all reasonable charges associated with providing this record. The HIPPA Privacy Rule does not prohibit the use, disclosure, or request of an entire medical record when authorized/requested by the patient.

VP My doctors are specifically instructed to discuss my condition and treatment of me with my attorney orally and provide him/her with written reports, as they may request.

VP Other _____

VP This authorization is limited to the following treatment: ***Unlimited***

***If the information to be disclosed contains any of the following types of information or records listed below, additional laws relating to the use and disclosure of this information may apply. I understand the federal or state law may restrict redisclosure of HIV/AIDS information, mental health information, genetic testing information and drug/alcohol diagnosis, treatment, or referral information. I understand and agree that this information will be disclosed if I place my initials in the applicable space next to the type of information:

VP ***HIV/AIDS information/records
VP ***Mental Health information/records
VP ***Genetic Testing information/records
VP ***Drug/alcohol diagnosis, treatment, or referral information: Any and all information of any kind of nature Federal Regulation, 42 CFR Part 2, requires a description of how much and what kind of information is to be disclosed.

Except to the extent that action has been taken in reliance of this authorization, I understand that I may revoke this authorization at any time by giving written notice to this provider. I understand that, if I revoke my authorization, any use or disclosure already made with my permission cannot be undone. **Unless revoked earlier, this authorization will expire in one year from the date signing or shall remain in effect for the period reasonably needed to complete the request.** To the extent an existing consent is more restrictive than this authorization, this authorization controls.

Refusal to sign this authorization will not affect my ability to obtain health care services or reimbursement for services. The only circumstance when refusal to sign means I will not receive health care service is if the health care service are solely for the purpose of providing health information to someone else and the authorization is necessary to make that disclosure. My refusal to sign this authorization does not adversely affect my enrollment in a health plan or eligibility for health benefits, unless the authorized information is necessary to determine if I am eligible to enroll in the health plan.

I AM WILLING THAT A DATED AND SIGNED PHOTOCOPY OF THIS AUTHORIZATION HAVE THE SAME FORCE AND EFFECT AS ORIGINAL.

I have reviewed and I understand this Authorization. I understand that, since my attorney is not a covered entity under federal privacy regulations, the information described above may be redisclosed and no longer protected by the HIPAA Privacy regulations. ***Specifically, I understand that the information used or disclosed pursuant to this Authorization may be subject to re-disclosure by my attorneys in the course of my claim and therefore may no longer be protected under Federal Law.***

4/30/2024
(Date) (Signature of patient/individual)

(Date) (Signature of person authorized by law)

CENTRAL WYOMING NEUROLOGY
a Memorial Hospital of Converse County Clinic

Central Wyoming Neurology EMG Lab
5820 East Second Street
Casper, WY. 82609
Tel: (307) 234-9037
Fax: (307) 234-9042

Patient: VIKTORIYA A PULLIAM
Patient ID: 24-190
Sex: Female
Date of Birth: 12/6/1989
Patient Age On Visit Date: 34 Years 3 Months Old
Examining Physician: SANTIAGO, M.D.
Referring Physician: SARA THRONBURG, NP-C
Notes: 34 year old female s/p cervical spine surgeries with neck pains with upper extremity paresthesias

Sensory NCS

Nerve / Sites	Rec. Site	Onset ms	Peak ms	NP Amp µV	PP Amp µV	Dist cm	Vel m/s
R Median - Dig II							
Wrist	II	2.50	3.10	35.0	37.4	14	56.0
L Median - Dig II							
Wrist	II	2.35	2.85	35.3	37.0	14	59.6
R Ulnar - Dig V							
Wrist	Dig V	2.40	2.95	37.8	40.7	14	58.3
L Ulnar - Dig V							
Wrist	Dig V	2.20	2.75	37.4	40.9	14	63.6
R Radial – Thumb							
Forearm	Thumb	1.40	1.75	38.9	42.4	10	71.4
L Radial – Thumb							
Forearm	Thumb	1.20	1.70	39.0	42.0	10	83.3

Motor NCS

Nerve / Sites	Rec. Site	Lat ms	Amp mV	Rel Amp %	Dist cm	Vel m/s
R Median - APB						
Wrist	APB	3.05	8.2	100	8	
Ref.		4.40	4.0			
Elbow	APB	6.95	7.8	96.6	22	56.4
Ref.						49.0

VIKTORIYA A PULLIAM 24-190 3/28/2024

Nerve / Sites	Rec. Site	Lat ms	Amp mV	Rel Amp %	Dist cm	Vel m/s
L Median – APB						
Wrist	APB	2.85	8.1	100	8	
Ref.		4.40	4.0			
Elbow	APB	6.70	7.7	98.1	22	57.1
Ref.						49.0
R Ulnar - ADM						
Wrist	ADM	2.50	9.8	100	8	
Ref.		4.40	8.0			50.0
B.Elbow	ADM	5.75	9.5	98.2	18	55.4
Ref.						49.0
A.Elbow	ADM	6.85	9.0	94.4		
Ref.						49.0
L Ulnar – ADM						
Wrist	ADM	2.60	9.9	100	8	
Ref.		4.40	8.0			50.0
B.Elbow	ADM	5.81	9.6	97.7	18	56.0
Ref.						49.0
A.Elbow	ADM	6.75	9.2	92.3		
Ref.						49.0

F Wave

Nerve	Fmin ms
R Median	34.10
R Ulnar	34.35
L Median	34.05
L Ulnar	34.20

EMG Summary Table									
	Spontaneous					MUAP			Recruitment
	IA	Fib	PSW	Fasc	H.F.	Amp	Dur.	PPP	Pattern
R. SupSpinat C5,6 SupScap	N	None	None	None	None	N	N	N	N
R. Rhomb Maj C5 DorsScap	N	None	None	None	None	N	N	N	N
R. Deltoid C5,6 Axillary	N	None	None	None	None	N	N	N	N
R. Triceps C6,7,8 Radial	N	None	None	None	None	N	N	N	N
R. Biceps C5,6, Musculocut	N	None	None	None	None	N	N	N	N
R. Pron Teres C6,7 Median	N	None	None	None	None	N	N	N	N
R. FirstDorsInt C8,T1 Ulnar	N	None	None	None	None	N	N	N	N
R. AbdPollBr C8,T1 Median	N	None	None	None	None	N	N	N	N
R. AbdDigMin C8,T1 Ulnar	N	None	None	None	None	N	N	N	N
L. SupSpinat C5,6 SupScap	N	1+	1+	None	None	N	N	N	N
L. Rhomb Maj C5 DorsScap	N	None	None	None	None	N	N	N	N
L. Deltoid C5,6 Axillary	N	1+	1+	None	None	N	N	N	N
L. Triceps C6,7,8 Radial	N	1+	None	None	None	N	N	N	N
L. Biceps C5,6, Musculocut	N	1+	1+	None	None	N	N	N	N
L. Pron Teres C6,7 Median	N	1+	1+	None	None	N	N	N	N
L. FirstDorsInt C8,T1 Ulnar	N	None	None	None	None	N	N	N	N
L. AbdPollBr C8,T1 Median	N	None	None	None	None	N	N	N	N
L. AbdDigMin C8,T1 Ulnar	N	None	None	None	None	N	N	N	N

VIKTORIYA A PULLIAM 24-190 3/28/2024

EMG Summary Table									
	Spontaneous					MUAP			Recruitment
	IA	Fib	PSW	Fasc	H.F.	Amp	Dur.	PPP	Pattern
Upper Cervical PSP	N	1+	1+	None	None	N	N	N	N
Middle Cervical PSP	N	1+	1+	None	None	N	N	N	N
Lower Cervical PSP	N	None	None	None	None	N	N	N	N

Summary: The nerve conduction study showed normal distal latencies, amplitudes and conduction velocities in the nerves studied. The median and ulnar F waves were normal. The concentric needle EMG study showed findings of denervation potentials with normal recruitment in the left C6 root innervated muscles.

Interpretation: Abnormal study. There was electrodiagnostic evidence for:

1. A left C6 radiculopathy;
2. There was no electrodiagnostic evidence for any brachial plexopathy or entrapment neuropathy.

Angelo M. Santiago, M.D.

CENTRAL WYOMING NEUROLOGY

a Memorial Hospital of Converse County Clinic

Patient: Pulliam,Viktoriya
DOB: 12/06/1989
Age/Sex: 34 / F
Admission Date: 03/28/24
Medical Record #: MR00140408
Account #: AM0000947362
Attending Provider: Angelo Santiago, MD

Procedure report

See Scanned document

Coding

Diagnosis Code(s): M54.16
Use EMG Codes (95860-95864, 95867-95870) when no NCS (95907-95913) performed same day.
Use 95885, 95886, 95887 for EMG services in conjuction w/NCS performed same day.
95886 - EMG Complete in conj w/NCS (5+ muscles, 3+ nerves, 4+ spinal levels): Yes (x 2)
95909 - NCS 5-6 Studies: Yes

<Electronically signed by Angelo Santiago, MD>
03/31/24 2034

<Signature on File>

cc:

5820 East Second Street, Casper WY 82609 www.ConverseHospital.com (307)234-9037 Phone (307)234-9042 Fax

MEMORIAL HOSPITAL
of Converse County
Advanced Medicine. Hometown Care.

Office Physician Progress Note

Patient: Pulliam,Viktoriya
DOB: 12/06/1989
Age/Sex: 34 / F
Admission Date: 04/02/24
Medical Record #: MR00140408
Account #: AM0000947531
Attending Provider: Sara Thronburg, FNP

History of Present Illness

Chief Complaint: Telephone Followup on Lab/Imaging

History of Present Illness/Details: Ms. Pulliam is a very pleasant 34 year old woman with chronic neck pain. She was in two separate car accidents, the first was in November 2019 and the second in February 2020. She states after these accidents she had some pain and numbness in her left arm. Ms. Pulliam underwent a C5-7 anterior cervical discectomy and fusion performed on 11/4/21. She states her neck felt great for about a year.

Ms. Pulliam presents via telehealth for a follow up. She reports stiffness in her neck and gets muscle twitching mostly at night. She reports weakness in bilateral arms and hands with the left being worse than the right. She states she has been dropping things and has difficulty opening a bottle. Ms. Pulliam presents a "bruising" type pain down her left arm. She has been taking ibuprofen for pain control. Ms. Pulliam states the weakness and muscle twitching seem to be getting worse. She did take a Medrol Dose pack, but states this did not help her symptoms at all.

Ms. Pulliam presents with a cervical myelogram CT and bilateral upper EMG/NCS to review.

Current Medications

Medication	Instructions	Recorded
ibuprofen 800 mg tablet	800 mg PO TID PRN tab	04/02/24
pantoprazole 40 mg tablet,delayed release	40 mg PO DAILY tab	04/02/24

Allergies

Allergy/AdvReac	Type	Severity	Reaction	Status	Date / Time
Penicillins	Allergy	Unknown	unknown	Verified	04/02/24 09:10

Telehealth Visit format
Patient visit performed via: Telephone
Telehealth Verbal Consent
1. I understand that this phone conversation is considered a clinical visit with my provider over the

Register Cliff RHC
650 W Whalen
Guernsey, WY 82214
(307)836-3009 Ph
(307)836-3022 Fx

Medical Office Building
700 E Center
Douglas, Wy 82633
(307)358-7300 Ph
(307)358-8498 Fx

Oregon Trail RHC
525 E Birch Street
Glenrock, WY 82637
(307)436-8838 Ph
(307)436-2476 Fx

Patient: Pulliam,Viktoriya
DOB: 12/06/1989
Age/Sex: 34 / F
Admission Date: 04/02/24

Medical Record #: MR00140408
Account #: AM0000947531
Attending Provider: Sara Thronburg, FNP

phone.
2. I authorize and consent to this verbal examination and will answer to the best of my ability any questions my provider deems necessary.
3. I agree to be financially responsible for the charges billed by Memorial Hospital of Converse County for this examination.
Verbal consent obtained: Yes
Telehealth Time Spent
Date of Service (start): 04/02/24
Start time: 09:05
Date of Service (stop): 04/02/24
Stop time: 09:24
Total time spent: 19

Visit Vitals

	04/02/24 09:06
Height	5 ft 5 in
Weight	160 lb
Body Mass Index (BMI)	26.6
Body Surface Area	1.84

Medical History (Reviewed 04/02/24 @ 09:06 by Mary Swanson)

Headache (Chronic)
Chronic back pain (Chronic)
Anxiety (Chronic)
COVID-19 (~11/2020)
History of ectopic pregnancy

Surgical History (Reviewed 04/02/24 @ 09:06 by Mary Swanson)

S/P cervical spinal fusion (~11/2021)

Family History (Reviewed 04/02/24 @ 09:06 by Mary Swanson)
High cholesterol
Father
Depression
Mother
Migraines
Mother
High blood pressure
Father

Social History (Updated 04/03/24 @ 11:56 by Eric Schubert)

Register Cliff RHC
650 W Whalen
Guernsey, WY 82214
(307)836-3009 Ph
(307)836-3022 Fx

Medical Office Building
700 E Center
Douglas, Wy 82633
(307)358-7300 Ph
(307)358-8498 Fx

Oregon Trail RHC
525 E Birch Street
Glenrock, WY 82637
(307)436-8838 Ph
(307)436-2476 Fx

Patient: Pulliam,Viktoriya
DOB: 12/06/1989
Age/Sex: 34 / F
Admission Date: 04/02/24

Medical Record #: MR00140408
Account #: AM0000947531
Attending Provider: Sara Thronburg, FNP

household members: spouse
marital status: married
current occupational status: employed
current occupation: Administration
Smoking and tobacco status: Former smoker
alcohol intake frequency: a few times a week

Exam

Details: Mild pain to palpation over the posterior cervical axial spine.
Bilateral upper extremity strength are equal with the expected section of weaker grip strength of the left hand more than the right. All other major muscle groups are 5/5.

No sensory deficits of the bilateral upper extremities.
Right upper extremity deep tendon reflex 2+
Left upper extremity deep tendon reflexes 1+.

Imaging Review/Electrodiagnostics
Imaging Review: I reviewed Ms. Pullliam's cervical myelogram with post-myelogram CT performed at WY-Summit Medical Center on 3/28/2024. The craniocervical junction is normal except for some mild osteoarthritis of the C1 arch/dens articulation. The cervical spinal cord is normal in morphology except for mild narrowing of the cord at the C5-6 level. There is 4 mm of tonsillar decent with no evidence of crowding of the foramen magnum. There are metallic interbody cages in the C5-6 and C6-7 interspaces with C5-7 anterior cervical plating. There appears to be solid fusion across the C5-6 and C6-7 interspaces. There is minimal to mild degenerative changes of the C2-3 and C3-4 interspaces, mild to moderate degenerative changes in the adjacent C4-5 interspace. There is mild loss of disc space height at the adjacent C7-T1 level. There are mild diffuse disc protrusions at C3-4 and C4-5, resulting in mild central canal stenosis at the latter level. There is mild to moderate adjacent level facet arthropathy at the adjacent C7-T1 level with no more then minimal to mild facet arthropathy at the remaining cervical levels. There is mild right C3-4 neural foraminal stenosis with no significant neural foraminal stenosis at any other surgical levels. There is good posterior alignment of the cervical vertebral bodies. There is straightening of the cervical lordosis.

Ms. Pulliam underwent 4 view cervical spine x-rays at elevation medical on January 18, 2024. There are postoperative changes of the anterior plate and screw fusion hardware construct involving C5-6 and C6-7, straightening of the cervical spine is seen small endplate osteophyte complex greatest involving C4-5. Mild to space narrowing at C4-5. No dynamic instability on flexion-extension is viewed on my read or radiology read.

Ms. Pulliam underwent a cervical MRI without contrast at elevation medical imaging. This was performed on January 18, 2024 ordered by Dr. Mansell. There is the C5-6 and C6-7 anterior cervical construct seen. There is a very mild levoscoliosis centered at the cervical thoracic junction. There is mild degenerative changes at C4-5. There is mild central canal stenosis noted at C3-4 and mild to moderate central canal stenosis at C4-5. There is also mild cord flattening due to diffuse disc protrusion resulting in mild left and moderate right neuroforaminal narrowing and facet arthropathy/

Register Cliff RHC
650 W Whalen
Guernsey, WY 82214
(307)836-3009 Ph
(307)836-3022 Fx

Medical Office Building
700 E Center
Douglas, Wy 82633
(307)358-7300 Ph
(307)358-8498 Fx

Oregon Trail RHC
525 E Birch Street
Glenrock, WY 82637
(307)436-8838 Ph
(307)436-2476 Fx

Patient: Pulliam,Viktoriya
DOB: 12/06/1989
Age/Sex: 34 / F
Admission Date: 04/02/24

Medical Record #: MR00140408
Account #: AM0000947531
Attending Provider: Sara Thronburg, FNP

hypertrophy worse on the right than the left. There is also mild cord flattening at C6-7 due to a protruding disc eccentric towards the left. Neuroforaminal stenosis is noted. There is cerebral tonsillitis projecting 6.5 mm inferior to the foramen magnum. This would need a separate brain MRI considering this is an incidental finding to exclude intracranial hypotension as cause for the low position of the cerebral tonsils. No syrinx of the cervical cord is seen. I have discussed all findings with patient.

Electrodiagnostics: I reviewed Ms. Pulliam's EMG/NCV studies of the bilateral upper extremities performed by Dr. Santiago on 3/28/2024. The study was interpreted as demonstrating electrodiagnostic evidence of a left C6 radiculopathy, with no evidence of brachial plexopathy or entrapment neuropathy.

Impression

1. Neck pain M54.2
2. History of cervical spinal arthrodesis;C5-C7 ACDF performed on 11/4/2021. Z98.1
3. Central stenosis of spinal canal; C4-5 and C7-T1 M48.00
4. Neuroforaminal stenosis of cervical spine M48.02
5. Protruded cervical disc; C4-5 and C7-T1 M50.20
6. Facet arthritis of cervical region M47.812
7. Degeneration of intervertebral disc of cervical region with osteophyte of cervical vertebra M50.30; M25.78
8. Bilateral arm weakness R29.898
9. Tingling of both upper extremities R20.2
10. Radicular pain of left upper extremity; feeling of bruising M79.2

Medical Decision Making

Ms. Pulliam has done extremely well until most recently, the last 5-6 months, when she started having persistent and progressive neck stiffness and body jerks along with bilateral upper extremity pain and paresthesias more significant on the left than the right. She did present back to us with a cervical spine MRI and x-ray series. Findings do show the postoperative changes at C5-6 and C6-7 with mild adjacent level degenerative changes at C3-4 and C7/T1 which includes mild disc protrusions and osteophytic spurring with disc osteophyte complexes. There is also mild flattening of the cord at those levels, per the MRI. There is an incidental finding of low position of the cerebral tonsils which would need to be worked up separately from her cervical spine which we have discussed.

We did discuss that 20% of the population can have degenerative changes above and below the fusion construct. Given the amount of artifact on the MRI I ordered a post myelogram CT and the EMG. Dr. Schubert and I discussed That from our standpoint the C5-6 and C6-7 levels are solidly fused with no significant stenosis and much decreased central canal stenosis and neural foraminal stenosis is seen. There are mild disc protrusions at C3-4, C4-5 and C7-T1 that do not produce on the myelogram any significant spinal canal stenosis. There is mild neuroforaminal stenosis at C3-4, and C4-5 according to the myelogram. There is no significant stenosis at C7-T1. The myelogram I explained does show us a better view without the artifact where the dye goes and does not go. I do feel that she does have some facet arthropathy/hypertrophy at C7-T1 more so on the left than the right.

At this time, Dr. Schubert and I discussed that we do not feel that her symptoms are coming from her cervical spine pathology that is really mild at this time. I spoke with Dr. Santiago this morning

Register Cliff RHC
650 W Whalen
Guernsey, WY 82214
(307)836-3009 Ph
(307)836-3022 Fx

Medical Office Building
700 E Center
Douglas, Wy 82633
(307)358-7300 Ph
(307)358-8498 Fx

Oregon Trail RHC
525 E Birch Street
Glenrock, WY 82637
(307)436-8838 Ph
(307)436-2476 Fx

Patient: Pulliam,Viktoriya
DOB: 12/06/1989
Age/Sex: 34 / F
Admission Date: 04/02/24
Medical Record #: MR00140408
Account #: AM0000947531
Attending Provider: Sara Thronburg, FNP

regarding her symptoms, and he did expect my call back regarding her. He is going to get her worked in sooner than later so that he can get her worked up from a neurological standpoint. He is happy to see her back I did discuss this in with Ms. Pulliam and she is amenable to this further workup with Dr. Santiago.

I would like to note for Dr. Santiago that she did have pretty significant lab work prior to her hysterectomy in November. I did instruct her to bring this to him when she follows up with him. She has not had her vitamin D tested recently. She does feel like this is progressive. She does report muscle twitching/jerking as well as the feeling of weakness in her bilateral legs when she is trying to go upstairs. She reports that it feels like growing pains in her arms left worse than right. She does report that her thyroid was deemed as normal but I do remember it being quite enlarged during her surgery.

Assessment & Plan

Problems

1. History of cervical spinal arthrodesis Z98.1
2. Degeneration of intervertebral disc of cervical region with osteophyte of cervical vertebra M50.30; M25.78
3. Facet arthritis of cervical region M47.812
4. Tingling of both upper extremities R20.2
5. Radicular pain of left upper extremity M79.2
6. Bilateral arm weakness R29.898
7. History of herpes zoster of eye Z86.19
8. Abnormal coordination R27.8
9. Weakness of both legs R29.898
10. Paresthesia and pain of both upper extremities R20.2; M79.601; M79.602
11. Muscle twitching R25.3

Plan - Sara Thronburg

1. Ms. Pulliam will be referred to Dr. Santiago for evaluation and treatment as indicated. -I will call Dr. Santiago to see if she can get him to evaluate her sooner than later.

This report was transcribed using Dragon Naturally Speaking voice recognition system. Despite editing, there may be grammatical and/or typographical errors in this transcription due to the limitations inherent with voice activated computer dictation.

Orders

Referrals:

Neurology Referral M79.601, M79.602, R20.2, R25.3, R27.8, R29.898

<Electronically signed by Sara Thronburg, FNP>
04/02/24 1018

Register Cliff RHC
650 W Whalen
Guernsey, WY 82214
(307)836-3009 Ph
(307)836-3022 Fx

Medical Office Building
700 E Center
Douglas, Wy 82633
(307)358-7300 Ph
(307)358-8498 Fx

Oregon Trail RHC
525 E Birch Street
Glenrock, WY 82637
(307)436-8838 Ph
(307)436-2476 Fx

Patient: Pulliam,Viktoriya
DOB: 12/06/1989
Age/Sex: 34 / F
Admission Date: 04/02/24

Medical Record #: MR00140408
Account #: AM0000947531
Attending Provider: Sara Thronburg, FNP

<Electronically signed by Eric Schubert >
04/03/24 1156
<Signature on File>

cc: Angelo Santiago, MD

Register Cliff RHC
650 W Whalen
Guernsey, WY 82214
(307)836-3009 Ph
(307)836-3022 Fx

Medical Office Building
700 E Center
Douglas, Wy 82633
(307)358-7300 Ph
(307)358-8498 Fx

Oregon Trail RHC
525 E Birch Street
Glenrock, WY 82637
(307)436-8838 Ph
(307)436-2476 Fx

MEMORIAL HOSPITAL
of Converse County
Advanced Medicine. Hometown Care.

Office Physician Progress Note

Patient: Pulliam,Viktoriya
DOB: 12/06/1989
Age/Sex: 34 / F
Admission Date: 02/13/24

Medical Record #: MR00140408
Account #: AM0000938501
Attending Provider: Sara Thronburg, FNP

History of Present Illness

Chief Complaint: Neck pain

History of Present Illness/Details: Ms. Pulliam is a very pleasant 34 year old woman with chronic neck pain. She was in two separate car accidents, the first was in November 2019 and the second in February 2020. She states after these accidents she had some pain and numbness in her left arm. Ms. Pulliam underwent a C5-7 anterior cervical discectomy and fusion performed on 11/4/21. She states her neck felt great for about a year.

Ms. Pulliam presents with returning neck pain. She states she has terrible days and ok days. She reports stiffness in her neck and gets muscle twitching mostly at night. She reports weakness in bilateral arms and hands with the left being worse than the right. She states she has been dropping things and has difficulty opening a bottle. Ms. Pulliam presents a "bruising" type pain down her left arm. She has been taking ibuprofen for pain control. She has not done any injections or therapy.

Ms. Pulliam presents with a recent cervical MRI and x-rays to review.

Current Medications

Medication	Instructions	Recorded
methylprednisolone 4 mg tablets in a dose pack	See Rx Instructions PO PER PKG DIR #21 tab	02/18/24

Allergies

Allergy/AdvReac	Type	Severity	Reaction	Status	Date / Time
Penicillins	Allergy	Unknown	unknown	Verified	02/01/22 09:47

Visit Vitals

	02/13/24 13:53
Height	5 ft 5 in

Register Cliff RHC
650 W Whalen
Guernsey, WY 82214
(307)836-3009 Ph
(307)836-3022 Fx

Medical Office Building
700 E Center
Douglas, Wy 82633
(307)358-7300 Ph
(307)358-8498 Fx

Oregon Trail RHC
525 E Birch Street
Glenrock, WY 82637
(307)436-8838 Ph
(307)436-2476 Fx

Patient: Pulliam,Viktoriya
DOB: 12/06/1989
Age/Sex: 34 / F
Admission Date: 02/13/24

Medical Record #: MR00140408
Account #: AM0000938501
Attending Provider: Sara Thronburg, FNP

Weight	160 lb
Body Mass Index (BMI)	26.6
Body Surface Area	1.84

Medical History (Reviewed 02/13/24 @ 13:53 by Mary Swanson)

Headache (Chronic)
Chronic back pain (Chronic)
Anxiety (Chronic)
COVID-19 (~11/2020)
History of ectopic pregnancy

Surgical History (Reviewed 02/13/24 @ 13:53 by Mary Swanson)

S/P cervical spinal fusion (~11/2021)

Family History (Reviewed 02/13/24 @ 13:53 by Mary Swanson)
High cholesterol
Father
Depression
Mother
Migraines
Mother
High blood pressure
Father

Social History (Updated 02/18/24 @ 18:50 by Sara Thronburg)
household members: spouse
marital status: married
current occupational status: employed
current occupation: Administration
Smoking and tobacco status: Former smoker
alcohol intake frequency: a few times a week

Exam

Details: Mild pain to palpation over the posterior cervical axial spine.
Bilateral upper extremity strength are equal with the expected section of weaker grip strength of the left hand more than the right. All other major muscle groups are 5/5.

No sensory deficits of the bilateral upper extremities.
Right upper extremity deep tendon reflex 2+
Left upper extremity deep tendon reflexes 1+.

Imaging Review/Electrodiagnostics

Register Cliff RHC
650 W Whalen
Guernsey, WY 82214
(307)836-3009 Ph
(307)836-3022 Fx

Medical Office Building
700 E Center
Douglas, Wy 82633
(307)358-7300 Ph
(307)358-8498 Fx

Oregon Trail RHC
525 E Birch Street
Glenrock, WY 82637
(307)436-8838 Ph
(307)436-2476 Fx

Patient: Pulliam,Viktoriya
DOB: 12/06/1989
Age/Sex: 34 / F
Admission Date: 02/13/24

Medical Record #: MR00140408
Account #: AM0000938501
Attending Provider: Sara Thronburg, FNP

Imaging Review: Ms. Pulliam underwent 4 view cervical spine x-rays at elevation medical on January 18, 2024. There are postoperative changes of the anterior plate and screw fusion hardware construct involving C5-6 and C6-7, straightening of the cervical spine is seen small endplate osteophyte complex greatest involving C4-5. Mild to space narrowing at C4-5. No dynamic instability on flexion-extension is viewed on my read or radiology read.

Ms. Pulliam underwent a cervical MRI without contrast at elevation medical imaging. This was performed on January 18, 2024 ordered by Dr. Mansell. There is the C5-6 and C6-7 anterior cervical construct seen. There is a very mild levoscoliosis centered at the cervical thoracic junction. There is mild degenerative changes at C4-5. There is mild central canal stenosis noted at C3-4 and mild to moderate central canal stenosis at C4-5. There is also mild cord flattening due to diffuse disc protrusion resulting in mild left and moderate right neuroforaminal narrowing and facet arthropathy/ hypertrophy worse on the right than the left. There is also mild cord flattening at C6-7 due to a protruding disc eccentric towards the left. Neuroforaminal stenosis is noted. There is cerebral tonsillitis projecting 6.5 mm inferior to the foramen magnum. This would need a separate brain MRI considering this is an incidental finding to exclude intracranial hypotension as cause for the low position of the cerebral tonsils. No syrinx of the cervical cord is seen. I have discussed all findings with patient.

Impression

1. Neck pain M54.2
2. History of cervical spinal arthrodesis;C5-C7 ACDF performed on 11/4/2021. Z98.1
3. Central stenosis of spinal canal; C4-5 and C7-T1 M48.00
4. Neuroforaminal stenosis of cervical spine M48.02
5. Protruded cervical disc; C4-5 and C7-T1 M50.20
6. Facet arthritis of cervical region M47.812
7. Degeneration of intervertebral disc of cervical region with osteophyte of cervical vertebra M50.30; M25.78
8. Bilateral arm weakness R29.898
9. Tingling of both upper extremities R20.2
10. Radicular pain of left upper extremity; feeling of bruising M79.2

Medical Decision Making

Ms. Pulliam has done extremely well until most recently when she started having persistent neck stiffness and body jerks along with bilateral upper extremity pain and paresthesias more significant on the left than the right. She did present today with a cervical spine MRI and x-ray series. Findings do show the postoperative changes at C5-6 and C6-7 with adjacent level degenerative changes at C3-4 and C7/T1 which includes disc protrusions and osteophytic spurring with disc osteophyte complexes. There is also mild flattening of the cord at those levels. There is an incidental finding of low position of the cerebral tonsils which would need to be worked up separately from her cervical spine which we have discussed.

At this time we did discuss that 20% of the population can have degenerative changes above and below the fusion construct. Given the amount of artifact on the MRI I would like her to undergo a cervical postmyelogram CT. Given the upper extremity pain and paresthesias as well as dropping items I would like her to undergo a bilateral upper extremity EMG/NCV study. I did review everything with her in detail. I did answer her questions to her satisfaction and she was amenable to this plan

Register Cliff RHC
650 W Whalen
Guernsey, WY 82214
(307)836-3009 Ph
(307)836-3022 Fx

Medical Office Building
700 E Center
Douglas, Wy 82633
(307)358-7300 Ph
(307)358-8498 Fx

Oregon Trail RHC
525 E Birch Street
Glenrock, WY 82637
(307)436-8838 Ph
(307)436-2476 Fx

Patient: Pulliam,Viktoriya
DOB: 12/06/1989
Age/Sex: 34 / F
Admission Date: 02/13/24
Medical Record #: MR00140408
Account #: AM0000938501
Attending Provider: Sara Thronburg, FNP

outlined above.

Assessment & Plan

Problems

1. Neck pain M54.2
2. History of cervical spinal arthrodesis Z98.1
3. Central stenosis of spinal canal M48.00
4. Neuroforaminal stenosis of cervical spine M48.02
5. Protruded cervical disc M50.20
6. Facet arthritis of cervical region M47.812
7. Degeneration of intervertebral disc of cervical region with osteophyte of cervical vertebra M50.30; M25.78
8. Bilateral arm weakness R29.898
9. Tingling of both upper extremities R20.2
10. Radicular pain of left upper extremity M79.2

Plan

1. Ms. Pulliam will undergo a cervical post myelogram CT.
2. Ms. Pulliam will undergo BUE EMG/NCV studies.
3. Ms. Pulliam will undergo a trial of a Medrol dose pack.
4. Ms. Pulliam will follow up separately regarding other non-car accident related findings.
5. Ms. Pulliam will follow up once she has completed her imaging studies.

This report was transcribed using Dragon Naturally Speaking voice recognition system. Despite editing, there may be grammatical and/or typographical errors in this transcription due to the limitations inherent with voice activated computer dictation.

Medications

New:

methylprednisolone (Medrol (Pak)) Take as directed 21 tabs 0RF

<Electronically signed by Sara Thronburg, FNP>
02/18/24 1850

<Signature on File>

cc:

Register Cliff RHC
650 W Whalen
Guernsey, WY 82214
(307)836-3009 Ph
(307)836-3022 Fx

Medical Office Building
700 E Center
Douglas, Wy 82633
(307)358-7300 Ph
(307)358-8498 Fx

Oregon Trail RHC
525 E Brch Street
Glenrock, WY 82637
(307)436-8838 Ph
(307)436-2476 Fx

CENTRAL WYOMING NEUROLOGY

a Memorial Hospital of Converse County Clinic

Patient: Pulliam,Viktoriya A
DOB: 12/06/1989
Age/Sex: 34 / F
Admission Date: 04/09/24
Medical Record #: MR00140408
Account #: AM0000962138
Attending Provider: Angelo Santiago, MD

Referring Provider
Sara Thronburg, NP-C

Primary Provider
Amy Hawk, NP-C

Nurse Intake notes
Patient here to establish care, c/o bilateral arm and leg weakness, L arm spasticity

Does pt use assistive devices: No Cane, No Walker, No Wheelchair, No Commode Chair, No Hospital Bed, No Respiratory assist device
Does the patient receive Hospice Care?: No
Chief Complaint/History Present Illness

Viktoriya Pulliam is a most pleasant 34-year-old right-handed Caucasian female, who was referred by Sara Thronburg, NP-C for neurological evaluation for progressively worsening symptoms of muscle weakness and 'muscle twitches'. She noticed these approximately about six months ago and had been worsening. The 'muscle twitches' could occur in the abdominal muscles, left arm, and left thigh. She also reports of dropping things easily, to the point that she needs to focus everything that she does. She denies any cranial neuro-symptomatology, changes in her energy, sphincter functions Thinking that these could be from dehydration, she had been drinking quite a bit of water.

She had cervical spine surgery by Dr. Eric Schubert 2021 and suspecting that these could be from her neck as she had cervical spine surgery, she had called Sara Thornburg, NP-C because of stiffness in her neck, some numbness and pain in her left forearm. She was given Medrol Dosepak without any help. She had a cervical myelogram, which was unexplanatory. She did have some mild left C6 radiculopathy on electrodiagnostic study that was done on March 28, 2024. She does report that she had been making some mistakes at work and also wonders whether this is from anxiety. She used to see a therapist in the past.

Interpreter Required: No
Allergies

Allergy/AdvReac	Type	Severity	Reaction	Status	Date / Time
Penicillins	Allergy	Unknown	unknown	Verified	04/09/24 09:22

Current Medications

Medication	Instructions	Recorded
ibuprofen 800 mg tablet	800 mg PO TID PRN tab	04/02/24
pantoprazole 40 mg tablet,delayed release	40 mg PO DAILY tab	04/02/24

5820 East Second Street, Casper WY 82609 www.ConverseHospital.com (307)234-9037 Phone (307)234-9042 Fax

Patient: Pulliam,Viktoriya A
DOB: 12/06/1989
Age/Sex: 34 / F
Admission Date: 04/09/24

Medical Record #: MR00140408
Account #: AM0000962138
Attending Provider: Angelo Santiago, MD

Visit Vitals

	04/09/24 09:22
Height	5 ft 5 in
Weight	150 lb
Body Mass Index (BMI)	25.0
Body Surface Area	1.78
Blood Pressure	129/97
Blood Pressure Location	Left Arm
Respiratory Rate	16
Pulse Rate	79
Pulse Ox	96
Oxygen Delivery Method	room air

Medical History (Updated 04/11/24 @ 06:22 by Angelo Santiago)

Other symptoms and signs involving cognitive functions and awareness (Chronic)
Muscle weakness (Chronic)
Fasciculations (Chronic)
Headache (Chronic)
Chronic back pain (Chronic)
Anxiety (Chronic)
HTN (hypertension)
Migraine
COVID-19 (~11/2020)
History of ectopic pregnancy

Surgical History (Updated 04/09/24 @ 09:24 by Sherrie Phillips)

Hx of hysterectomy
Hx of sinus surgery
S/P cervical spinal fusion (~11/2021)

Family History (Updated 04/09/24 @ 09:25 by Sherrie Phillips)
No pertinent family history
Sister
High cholesterol
Father
Depression
Mother
Migraines
Mother
High blood pressure
Father

Social History (Updated 04/11/24 @ 06:28 by Angelo Santiago)

5820 East Second Street, Casper WY 82609 www.ConverseHospital.com (307)234-9037 Phone (307)234-9042 Fax

Patient: Pulliam,Viktoriya A
DOB: 12/06/1989
Age/Sex: 34 / F
Admission Date: 04/09/24
Medical Record #: MR00140408
Account #: AM0000962138
Attending Provider: Angelo Santiago, MD

household members: spouse
housing: house
marital status: married
number of children: 2
education level: college
current occupational status: employed
current occupation: Administration
caffeine: Yes (1-2)
Smoking and tobacco status: Former smoker quit date: 01/01/07
alcohol intake frequency: holidays/special occasions only
substance use type: does not use

Review of Systems

General
Full 14 point review of systems is unremarkable except as noted.

The past medical and surgical history, medications and allergies, social history, family history, review of systems were all reviewed by me for today's evaluation and were updated where necessary and discussed with the patient where pertinent.
Current: Headache, Forgetfulness, Loss of weight
EENT
Current: Sinus Problems
GI
Current: Nausea/Vomiting, Diarrhea, Constipation
Musc/Jnt/bn
Current: Pain (Arms and Legs), Weakness (Arms and Legs)

Exam

General
Examined in presence of our medical assistant, Sherrie Phillips, MA

General: Afebrile, Oriented x4, no acute distress and well groomed
Nutritional Appearance: mesomorphic body habitus
HENMT
Head: atraumatic and normal to inspection
Eyes
Periorbital: periorbital findings normal
Eyelids: eyelids normal
Conjunctivae: conjunctivae normal
Neck
General: no lymphadenopathy
Carotids: Yes no appreciable bruits
Chest
Chest: normal inspection of the chest
Resp

5820 East Second Street, Casper WY 82609 www.ConverseHospital.com (307)234-9037 Phone (307)234-9042 Fax

Patient: Pulliam,Viktoriya A
DOB: 12/06/1989
Age/Sex: 34 / F
Admission Date: 04/09/24
Medical Record #: MR00140408
Account #: AM0000962138
Attending Provider: Angelo Santiago, MD

Effort: normal respiratory effort
Auscultation: Yes clear to auscultation bilaterally
Cardio
Rate & Rhythm: regular rate and rhythm
Abdomen
Palpation: soft
Spine
Cervical: normal AROM and Lhermitte's sign negative
Thoracic: normal to inspection
Lumbar: lumbar ROM
Extrem
General: normal to inspection and no clubbing, cyanosis nor edema
Neurology
General: No apparent agitation, appropriate mood and interactive and pleasant
Mental Status: Yes grossly intact
Speech: fluent without appreciable dysarthria
Cranial Nerves: Grossly normal.
Coordination: Abnormal finger-to-nose test, Abnormal heel-to-shin test, Abnormal rapid alternating movement, Abnormal finger/toe tapping, Abnormal appreciable dysmetria, Abnormal appreciable dysdiadochokinesia or Abnormal other
Motor: strength 5/5 throughout (upper/lower ext) and no pronator/parietal drift
Strength: There was notable decreased bulk in her left biceps, which she expressed concern about. I did not appreciate any fasciculations.
Sensory: other (? Sensory level at T6)
DTR's: **Rt Triceps:** 2+, **Lt Triceps:** 2+, **Rt Biceps:** 2+, **Lt Biceps:** 2+, **Rt Brachioradialis:** 1+, **Lt Brachioradialis:** 1+, **Rt Patellar:** 3+, **Lt Patellar:** 3+, **Rt Ankle:** 1+, **Lt Ankle:** 1+
Gait: normal
Balance: Negative Romberg testing, Negative Pull through testing, Negative 25' timed walk

Assessment & Plan

Problems
1. Other symptoms and signs involving cognitive functions and awareness R41.89
2. Muscle weakness M62.81
3. Fasciculations R25.3
4. Anxiety F41.9

Plan
Progressively worsening symptoms of predominantly left sided weakness, cognitive concerns, query sensory level at T7 with symptoms suggestive of fasciculations. We will have her undergo an MRI of her brain to determine a primary demyelinating disorder, e.g. multiple sclerosis and check some blood tests including CBC, CMP, ESR, ANA, TSH, GAD, CPK, and aldolase. Consider spinal cord MRI studies should the brain MRI show findings of a demyelinating condition. We will make further recommendations thereafter.

Plan Detail
Additional Comments
This visit was 60 minutes including seeing the patient myself reviewing available records, discussing

5820 East Second Street, Casper WY 82609 www.ConverseHospital.com (307)234-9037 Phone (307)234-9042 Fax

Patient: Pulliam,Viktoriya A
DOB: 12/06/1989
Age/Sex: 34 / F
Admission Date: 04/09/24

Medical Record #: MR00140408
Account #: AM0000962138
Attending Provider: Angelo Santiago, MD

the assessment and plan with the patient and documenting the visit.
She expressed understanding and agreement with this plan and was encouraged to contact us should there be any questions or concerns in the interim.

<Electronically signed by Angelo Santiago, MD>
04/11/24 0628

<Signature on File>

cc:

5820 East Second Street, Casper WY 82609 www.ConverseHospital.com (307)234-9037 Phone (307)234-9042 Fax